

1. Administrative & Study Identification

Full Study Title: A Phase II, Open-label, Multicenter, Master Protocol to Evaluate the Safety and Efficacy of Novel Study Interventions and Combinations in Participants With Colorectal Cancer (CANTOR) **Brief Study Title:** A Study of Novel Study Interventions and Combinations in Participants With Colorectal Cancer **Short Title/Acronym:** CANTOR **Protocol Number:** D798VC00001 **NCT Number:** NCT06792695 **Posting ID:** 285947927338156627 **Trial Status:** Recruiting **Sponsor / Company Name:** AstraZeneca **Investigational Product:** Volrustomig (MEDI5752) **Phase of Development:** Phase II **Medical Monitor:** AstraZeneca Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the safety, tolerability, and anti-tumor activity (specifically Progression-Free Survival [PFS]) of novel study interventions and combinations in participants with Colorectal Cancer (CRC). **Secondary Objectives:** To evaluate Overall Survival (OS), Objective Response Rate (ORR), Disease Control Rate (DCR), and Duration of Response (DOR).

2.2 Background & Rationale To address the gap in therapeutic efficacy for patients with Mismatch-repair-proficient (pMMR) / Microsatellite stable (MSS) metastatic colorectal cancer (mCRC) in the absence of liver metastases. This Phase II master protocol evaluates whether adding the investigational drug volrustomig (a novel immunotherapy) to the standard of care FOLFIRI plus bevacizumab regimen is safe and provides superior anti-tumor activity compared to FOLFIRI and bevacizumab alone for adults who have not received prior systemic treatment for their metastatic disease.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional (Platform Master Protocol / Modular Study) **Trial Status:** Recruiting **Control Type:** Active Comparator (FOLFIRI + bevacizumab) **Blinding:** Open-label **Randomization:** Randomized

3.2 Planned Enrollment & Duration Target **\$N\$:** 120 participants **Number of Sites:** Multicenter, global trial **Estimated Study Start:** 12-Mar-2025 **Estimated Primary Completion:** 31-May-2027

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Histopathologically confirmed colorectal adenocarcinoma with the presence of measurable disease by RECIST 1.1 criteria. 2. Known pMMR/MSS status (only pMMR/MSS mCRC allowed) with no radiological evidence of liver metastasis. 3. ECOG performance status of 0 or 1, with a life expectancy $\geq$ 12 weeks at the time of screening.

4.2 Exclusion Criteria 1. Central nervous system metastases or spinal cord compression. 2. Potentially resectable disease with a multidisciplinary plan for radical surgery. 3. Active or prior documented autoimmune or inflammatory disorders, cardiac conditions, or history of deep venous thrombosis/pulmonary embolism.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Participants will be randomized to Arm A (Volrustomig + FOLFIRI + bevacizumab) or Arm B (FOLFIRI + bevacizumab). FOLFIRI consists of irinotecan (ATC Code: L01CE02; Trade names: Camptosar, Onivyde), 5-FU, and leucovorin. **Route of Administration:** Intravenous (IV) infusion for all study drugs (volrustomig, FOLFIRI components, and bevacizumab). **Duration of Treatment:** Cycles administered until disease progression, unacceptable toxicity, or completion of the study.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care measures for chemotherapy-related toxicities. **Prohibited:** Prior exposure to immune-mediated therapy or any prior systemic therapy for mCRC (except for neoadjuvant/adjuvant chemotherapy completed $\geq$ 6 months prior).

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, FFPE tumor sample collection, RECIST 1.1 Baseline Scans
Baseline	Day 0	Randomization, First Dose of Volrustomig/FOLFIRI/Bevacizumab, Vital Signs
Interim	Treatment Cycles	Safety Labs, Treatment Administration, RECIST 1.1 Efficacy Assessments, AE Monitoring
End of Study	Follow-up	Final Vital Signs, Exit Interview, Survival Follow-up

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Progression-Free Survival (PFS) and Incidence of treatment-emergent adverse events (safety and

tolerability). **Measurement Method:** Radiological assessments according to RECIST 1.1 criteria and standard CTCAE toxicity grading.

7.2 Secondary & Exploratory Endpoints * Overall Survival (OS) * Objective Response Rate (ORR) * Disease Control Rate (DCR) * Duration of Response (DOR)

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring of AEs graded by CTCAE v5.0 (specifically tracking unresolved toxicity Grade ≥ 2 from previous therapies and immune-related AEs). **Serious Adverse Events (SAEs):** Reported within 24 hours of site awareness, particularly concerning hypertensive crises, GI perforations, and fistulae. **Data Monitoring Committee (DMC):** Independent Data Monitoring Committee to evaluate safety signals across the platform's substudies.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for all efficacy endpoints (PFS, OS, ORR). Safety Set for all participants who receive at least one dose of the study intervention. **Sample Size Justification:** $N = 120$, appropriately powered to detect meaningful efficacy signals in the substudy evaluating volrustomig combinations. **Handling of Missing Data:** Standard oncology trial conventions for censoring PFS and OS based on the last known date alive or progression-free.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Regular onsite and remote monitoring visits by AstraZeneca or its designees. **Audit Trail:** Electronic Case Report Form (eCRF) tracking with source data verification for key safety and efficacy endpoints (e.g., RECIST 1.1 measurements).

11. Study Identifiers & Governance

Primary ID: D798VC00001 **Registry Number:** NCT06792695 **Posting ID:** 285947927338156627 **Sponsor / Company Name:** AstraZeneca **Study Title:** CANTOR **Official Regulatory Title:** A Phase II, Open-label, Multicenter, Master Protocol to Evaluate the Safety and Efficacy of Novel Study Interventions and Combinations in Participants With Colorectal Cancer (CANTOR)

12. Clinical Framework (Colorectal Cancer Specific)

Primary Disease Area: Metastatic Colorectal Cancer (mCRC)
Diagnosis Threshold: Histopathologically confirmed colorectal adenocarcinoma, pMMR/MSS
Medical Methodology: Addition of an investigational immunotherapy (volrustomig) to standard-of-care chemotherapy/targeted therapy (FOLFIRI + bevacizumab)
Optimization Target: Progression-Free Survival (PFS) and Disease Control

13. Study Procedures & Methodology

Management Pathway: Standard IV infusion pathway for FOLFIRI + bevacizumab, with or without volrustomig
Education Protocol (HCP): Protocol-specific training on immune-mediated adverse event management and RECIST evaluation.
Education Protocol (Patient): Informed consent process detailing the risks of multi-drug combination therapies (e.g., GI perforations, thrombosis).
Quality Audit Mechanism: Centralized imaging review mechanisms for RECIST 1.1 and standardized AE reporting protocols.

14. Observation & Follow-Up Schedule

Total Duration: Follow-up until estimated study completion in Jun 2028
Onsite Visit Cadence: Aligned with chemotherapy cycles (e.g., Q2W or Q3W depending on standard FOLFIRI dosing).
Remote Monitoring Frequency: Continuous survival follow-up after progression.
Checklist Review Interval: Prior to every infusion cycle.

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]				